

RIVERBEND FAMILY MEDICINE

1420 Mill Creek Road, Suite 200

PROGRESS NOTE

Patient ID: RB-884213

Age: 58 Sex: Male

Date of Service: 03/14/2026

Place of Service: 11

Payer: Meridian Health Plan

Provider: Alan T. Whitfield, MD NPI: 1932847561

CHIEF COMPLAINT:

Follow-up of diabetes and knee pain.

HISTORY OF PRESENT ILLNESS:

Established patient returns for routine follow-up. He reports his home glucose readings have been running in the 180s to 220s over the past two months. He has been taking metformin 1000 mg twice daily and states he has not missed doses. He describes ongoing burning and numbness in both feet in a stocking-glove distribution, worse at night, which has progressed since his last visit.

He also reports persistent right knee pain, worse with stairs and prolonged standing, partially relieved by acetaminophen. He denies chest pain, denies shortness of breath, and denies any fever or chills. There is no evidence of an acute infection. He continues to smoke approximately half a pack of cigarettes per day and has done so for thirty years.

PAST MEDICAL HISTORY:

Type 2 diabetes mellitus. Essential hypertension. Hyperlipidemia.

FAMILY HISTORY:

Father with obstructive sleep apnea. Mother deceased, stroke.

SOCIAL HISTORY:

Lives with spouse. Works as a warehouse supervisor. Current smoker.

MEDICATIONS:

Metformin 1000 mg PO BID

Lisinopril 20 mg PO daily

Atorvastatin 40 mg PO daily

ALLERGIES:

No known drug allergies.

REVIEW OF SYSTEMS:

Constitutional: negative for fever. Cardiovascular: negative for chest pain.

Respiratory: negative for cough. Neurologic: positive for bilateral foot numbness.

VITAL SIGNS:

BP 142/88 HR 78 Temp 98.2 F Wt 214 lb BMI 31.6

PHYSICAL EXAMINATION:

General: well-appearing, no acute distress.
Cardiovascular: regular rate and rhythm, no murmur.
Respiratory: clear to auscultation bilaterally.
Extremities: right knee with medial joint line tenderness and a small effusion.
Crepitus with range of motion. No warmth or erythema.
Neurologic: decreased monofilament sensation bilaterally to the mid-foot.

LABORATORY:

Reviewed labs from 03/10/2026. Hemoglobin A1c 8.9%. Creatinine 1.0.
Ordered a comprehensive metabolic panel and a hemoglobin A1c for three months.

ASSESSMENT AND PLAN:

1. Type 2 diabetes mellitus - poorly controlled on current therapy with an A1c of 8.9%, up from 7.8%. Increased metformin is not an option at maximum dose. Will start empagliflozin 10 mg daily. Reviewed hypoglycemia precautions.
2. Diabetic neuropathy - symptoms have progressed and now interfere with sleep. Start gabapentin 300 mg at bedtime, titrate as tolerated. Foot care reviewed in detail.
3. Essential hypertension - blood pressure above goal today at 142/88. Continue lisinopril 20 mg daily and recheck in six weeks.
4. Knee osteoarthritis - imaging from last year showed medial compartment narrowing. Arthrocentesis knee with corticosteroid injection performed today in the right knee. See procedure note below.
5. Nicotine dependence - counseled on cessation. Discussed nicotine replacement and set a quit date of 04/01/2026. Approximately 6 minutes spent on cessation counseling.

PROCEDURE:

Arthrocentesis knee, right. After informed consent and sterile prep, the right knee was entered via a lateral approach. 8 mL of clear yellow fluid was aspirated. Methylprednisolone acetate 40 mg was then injected. The patient tolerated the procedure well. No complications.

Total encounter time including the procedure: 40 minutes.

Electronically signed by Alan T. Whitfield, MD on 03/14/2026 at 16:22.